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The Pandemic's Sleeping Giant: The Vaccine

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Through the seven long months that COVID-19 has run rampant in North America, we have seen many waves of supply and demand of varying degrees come and go. Through periods where politicians were urging us to buy hand sanitizers that were constantly sold out, to toilet paper becoming a hot commodity – leaving people to forget that tissues or a shower are probably perfect substitutes to Cottonelle or Charmin – this idea of scarcity has become a greater focus for many that have not yet experienced it in their lifetime.

Policymakers, consumers, and organizations alike have begun to recognize the importance of preventing the spread of COVID-19. Many governments have been allocating money towards research, vaccine trials, and COVID-19 testing. Universities have accepted recommendations from public health officials and opted into online learning, taking a huge hit to their revenue in doing so. More so than ever in recent years, individuals have had to make tough sacrifices in the midst of this pandemic.

It's evident that all of the actions and decisions made by the actors in society have contributed in showing the increasing value of individual health. Many believe that a vaccine would be the end-all to the components of life that COVID-19 has made so difficult, counting on it to eradicate not only the virus, but the masks, social distancing, and all other components that have followed.

Though there is another dark component in eradicating the virus lingering in the corner that will place tremendous pressure on the healthcare system. Behind the scenes, policy experts are quietly devising a plan on the logistics of a COVID-19 vaccine administration. And, something weighing heavily on their minds is whether or not they should save a billionaire, ER doctor, or a 90-year-old grandma in a nursing home first.

The near future is trade-off plentiful

The vaccine distribution plan will not be the government's first rodeo when it comes to trade-offs during the pandemic. Hospitals have suspended certain non-COVID related services to accommodate the different needs arising from the virus (1). Federal and provincial governments have had to mandate business shutdowns to help slow the spread, ultimately impacting economic growth (2).

There has been a trade-off in every single decision, policy, or political campaign – but the difference lies in that it is human lives at stake, rather than arbitrary financial percentages and monetary figures.

While not everyone will agree with whatever distribution policy is put in place, it's safe to say that a viable, efficient form of distribution may be one in which health care workers are first in line for the scarce vaccine. Although Canada is faced with tough options, they can look to international counterparts for advice. The Center for Disease Control and Prevention (CDC) has proposed for the distribution of vaccines in the United States.

The first batch of recipients would be 12 million of the most essential health, national security, and other essential workers (4). The next of the vaccines would go to the 110 million people deemed most at risk, a large umbrella statement for the elderly, those that live in long-term care facilities, and other essential workers (5). The general public would follow only once there is a proven, sustainable supply of the vaccine (6) – and just with a lot of things between Canada and the United States, it's likely that our distribution plan can be expected to be similar.

In Canada, we have seen the need for ensuring that our front-line medical staff are protected, so that they can in turn treat others infected with the virus. As of July 2020, Canada has already seen 21,842 health care workers contract the virus, with the number sure to grow alongside the growing number of cases (3).

Figure 1. cases and deaths among health care workers in Canada as of July 23, 2020. Retrieved from Canadian Institute for Health Information (2020)

Some argue that the best distribution policy would be one that targets the super-spreaders of the vaccine – children to 34 year-olds (7), who occupy the largest demographic of COVID-19 positive cases, but typically suffer the least symptoms. Although targeting the largest cohort could slow down the spread of the virus, this group of people usually suffer the least COVID-19 related hardships and would not require the vaccine as urgently as those that are immune compromised.

While the Public Health Agency has yet to comment on a plan for Canada, it's likely that their approach would remain similar to that of their neighbours to the south. It will be interesting to watch not only to see how Canada distributes by demographic, but also by geographic region. There is a very large discrepancy between the population densities of the two countries, just as case numbers do between Canada's provinces, territories, and cities. This could mean deciding whether to prioritize larger cities, where social distancing is more difficult, such as Toronto, or to prioritize the hardest hit province, Quebec.

It would appear that this plan by the CDC would be in the best interests of society at large, a fundamental assumption of economics underlines a potential calamity: self-interest.

Scarcity, Demand, and Morality

With any vaccine distribution plan, a moral trade-off is bound to occur. We have seen moral theory come into play throughout the pandemic, and the theory of utilitarianism is one that has been and will continue to be heavily prevalent in any COVID-19 related public policy.

The ideology of utilitarianism is one that assures that the most ethical choice is one that will produce the greatest good for the largest number of people. Here, it could be argued that front line medical staff must be protected first, so that they can treat others that are suffering from the virus – mimicking this idea of the greatest good for the largest number of people.

While, arguably, government policies for a society may be heavily rooted in utilitarianism, the same cannot be said for the individuals that live within that society. Our experiences with the 2009 H1N1 vaccine demonstrates that a chaotic, anti-utilitarian situation is very, very possible.
The distribution plan for H1H1 was quite different from ones proposed for COVID-19 in neighbouring countries. In Canada specifically, the first cohort of people designated to receive the vaccine were children, as they typically suffered the most serious effects (8).

Despite this, the facts and policies alone were not enough to stop the 2009 athlete H1N1 scandal in Canada. The Toronto Raptors were able to skip vaccine queues where there were not even enough vaccines to vaccinate all school-aged children (9) – at the time, Ontario had only enough vaccine resources to immunize 2.2 million people out of the 3.4 million that fell in their priority group (10). Employees were also fired in Alberta for allowing the Calgary Flames players to be vaccinated behind closed doors (1)1. With this large bottle-neck in future distributions, and larger quantities demanded for the COVID-19 vaccine, we can only hope that the rich and powerful do not consider their own moral trade-off, using their deep pockets to their advantage.

While this may seem like a far-out idea, we are already seeing this occur on the global stage. Countries that can afford it have been pre-purchasing far more vaccines than those that are less economically sound – those that can afford it buy out more of the supply. Without price controls, these less economically powerful countries are unlikely to be able to access the number of vaccines required to protect their population. We watched this specifically play out during the AIDS crisis, where big pharma went at lengths to sue South Africa for enacting legislation to make the drugs more affordable (12).

Conclusion

There is no doubt that the choice of prioritizing specific demographics in society over others will be an immense decision on public health officials. There will evidently be tremendous pressure on policymakers to ensure that their choices minimize the trade-offs and negative externalities that will occur.

With such a larger degree of scarcity in supply and abundance of demand, the issues of morality and ethics become much more prevalent. The Canadian scandal with professional athletes skipping the queue for H1N1 vaccines demonstrated that whatever distribution plan the government decides to adopt, it must be heavily enforced and not subjected to bribery offenses.

Admittingly so, while planning is critical to ensuring the success of a vaccine distribution plan, many policy planning efforts are based heavily on the assumption that the millions of vaccines that have been pre-purchased will be effective in stopping the spread of COVID-19.

Like all things, there is no one size fits all.

Endnotes

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